

CHILD BEHAVIOR CHECKLIST (CBCL)

COMPLETE GUIDE

For MPhil Clinical Psychology Trainees

TABLE OF CONTENTS

1. Introduction & Historical Background
2. Nature of the Test
3. Purpose & Applications
4. Versions of the CBCL
5. Structure & Content
6. Administration Procedure
7. Scoring System
8. Syndrome Scales (Narrow-Band)
9. Broad-Band Scales (Internalizing/Externalizing)
10. DSM-Oriented Scales
11. Competence Scales
12. T-Score Interpretation & Clinical Cutoffs
13. Interpretation Guidelines
14. Report Writing Format
15. Sample Report Template
16. Psychometric Properties
17. Advantages & Limitations
18. References
- Appendix A: Quick Reference T-Score Cutoffs
- Appendix B: Item-to-Scale Assignment Summary
- Appendix C: Clinical Tips for Interpretation
- Appendix D: Comparison of CBCL Versions

1. INTRODUCTION & HISTORICAL BACKGROUND

1.1 Developer & Origin

The Child Behavior Checklist (CBCL) was developed by Thomas M. Achenbach and Craig Edelbrock in 1983 at the University of Vermont, Department of Psychiatry, Burlington, Vermont, USA.

The instrument was substantially revised in 2001 by Thomas M. Achenbach and Leslie A. Rescorla, incorporating DSM-oriented scales and updated norms. This revision forms the current standard version in clinical use.

1.2 The ASEBA System

The CBCL is the flagship instrument of the Achenbach System of Empirically Based Assessment (ASEBA) -- a comprehensive, multi-informant assessment system designed to evaluate behavioral, emotional, and social functioning across the lifespan. ASEBA re

1.3 Historical Development Timeline

Year	Development
1966	Achenbach's initial factor-analytic studies of child behavior problems
1978	First edition of the Child Behavior Checklist published
1983	Publication of the formal manual (Achenbach & Edelbrock) -- established the standard
1991	Major revision with updated norms and revised syndrome scales
2001	Current revision (Achenbach & Rescorla) -- added DSM-oriented scales, updated norms
2007	Multicultural supplement with multi-society norms
2014+	Continued cross-cultural validation across 100+ societies

1.4 Theoretical Foundation

Unlike many psychological instruments that are derived from a specific theoretical model (e.g., psychoanalytic, cognitive-behavioral), the CBCL is empirically based. This means:

- Items were derived from clinical literature review, clinical records, and consultation with parents and clinicians -- not from a priori theoretical categories
- Scales were constructed through multivariate statistical methods (principal components analysis, factor analysis) applied to large datasets of real clinical referrals
- The taxonomy of syndromes emerged from the data itself rather than being imposed from existing diagnostic frameworks
- This "bottom-up" approach ensures that the instrument captures patterns of co-occurring problems as they actually present in children, rather than as theory predicts they should

1.5 Key References

1. Original Manual: Achenbach, T.M., & Edelbrock, C. (1983). Manual for the Child Behavior Checklist and Revised Child Behavior Profile. Burlington, VT: University of Vermont, Department of Psychiatry.
2. Current Revision: Achenbach, T.M., & Rescorla, L.A. (2001). Manual for the ASEBA School-Age Forms & Profiles. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.
3. Multicultural Norms: Achenbach, T.M., et al. (2007). Multicultural Supplement to the Manual for the ASEBA School-Age Forms & Profiles. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.

2. NATURE OF THE TEST

2.1 Classification

Parameter	Description
Type	Parent-report standardized behavioral checklist
Category	Empirically-based assessment (not theory-driven)
Approach	Quantitative -- yields T-scores and percentile ranks
Format	Paper-and-pencil / Computer-administered
Level	Level B (requires supervised training in psychometrics)
Response Format	3-point Likert scale: 0, 1, 2
Time Frame Assessed	Past 6 months
Administration Time	15-20 minutes
Scoring Time	10-15 minutes (hand-scoring); instant (computer)

2.2 Item Structure

Component	Details
Total items (CBCL/6-18)	113 problem behavior items + 20 competence items
Response scale	0 = Not True (as far as you know) 1 = Somewhat or Sometimes True 2 = Very True or Often True
Time frame	Based on the preceding 6 months
Open-ended items	3 items allow parents to add behaviors not listed

2.3 Target Population

Version	Age Range	Informant
CBCL/6-18	6-18 years	Parent or parent-surrogate
CBCL/1.5-5	1.5-5 years	Parent or parent-surrogate

2.4 Who Can Administer

- Administration: No special qualifications needed -- any literate adult can fill it out
- Scoring & Interpretation: Requires a trained clinical psychologist, psychiatrist, or supervised trainee with knowledge of psychometrics, child development, and psychopathology
- Report Writing: Should be conducted by or under the supervision of a licensed mental health professional

2.5 Companion Instruments (Multi-Informant System)

The CBCL is designed to be used as part of a multi-informant system:

Instrument	Informant	Age Range
CBCL/6-18	Parent	6-18 years
TRF/6-18 (Teacher's Report Form)	Teacher	6-18 years
YSR/11-18 (Youth Self-Report)	Youth (self)	11-18 years
CBCL/1.5-5	Parent	1.5-5 years
C-TRF/1.5-5 (Caregiver-Teacher Report Form)	Caregiver/Teacher	1.5-5 years

Clinical Note: Ideally, obtain ratings from multiple informants (both parents + teacher + youth if ≥ 11 years) to examine cross-informant agreement and discrepancies. Research consistently shows moderate informant agreement ($r \sim .60$ for similar infor

3. PURPOSE & APPLICATIONS

3.1 Primary Purposes

1. Comprehensive Assessment: Quantify the range and severity of behavioral/emotional problems and social competencies in children and adolescents
2. Clinical Screening: Identify children who may need clinical services or further diagnostic evaluation
3. Treatment Planning: Establish baseline problem profiles to guide intervention selection
4. Outcome Monitoring: Track changes in problem scores over time to evaluate treatment effectiveness
5. Multi-Informant Comparison: Compare perceptions across parents, teachers, and youth to identify context-specific or pervasive problems

3.2 Clinical Applications

| Application | How CBCL Is Used |
| Initial Clinical Assessment | Provides standardized baseline of problem behaviors and competencies at intake |
| Differential Screening | Pattern of elevated scales helps generate hypotheses about internalizing vs. externalizing vs. mixed pr
| Treatment Monitoring | Repeated administrations (e.g., every 3-6 months) document treatment response |
| Program Evaluation | Agencies use aggregate CBCL data to evaluate service effectiveness |
| Forensic/Custody Evaluations | Documents parent perceptions of child behavior; cross-parent comparisons reveal agreement/discrep
| School-Based Assessment | TRF companion form extends assessment to school setting |
| Research | Gold-standard measure in child psychopathology research worldwide |

3.3 Research Applications

- Epidemiological studies of childhood mental health prevalence
- Cross-cultural comparison of child behavior problems
- Longitudinal studies of developmental psychopathology
- Treatment outcome research (randomized controlled trials)
- Genetic and neurobiological studies of behavior
- Risk and resilience research
- Validation studies of other instruments

3.4 Settings Where CBCL Is Used

- Child and adolescent psychiatric clinics
- Pediatric hospitals and primary care
- School psychology services
- Community mental health centers
- Child welfare and protection services
- Juvenile justice settings
- Private practice (clinical psychology, psychiatry)
- Research institutions and universities

4. VERSIONS OF THE CBCL

4.1 Complete ASEBA Forms

Form	Age Range	Items	Informant	Key Features
CBCL/6-18	6-18 years	113 problem + 20 competence	Parent	Primary school-age parent form; 8 syndrome scales + DSM scales
CBCL/1.5-5	1.5-5 years	99 problem + 1 language item	Parent	Preschool parent form; 7 syndrome scales
TRF/6-18	6-18 years	113 problem + adaptive items	Teacher	Parallel to CBCL/6-18; includes academic & adaptive scales
YSR/11-18	11-18 years	112 problem + competence	Youth (self)	Self-report version; requires 5th-grade reading level
C-TRF/1.5-5	1.5-5 years	99 problem items	Caregiver/Teacher	Daycare/preschool teacher form

4.2 Version History

Edition	Year	Key Changes
Original CBCL	1978/1983	First standardized form; age- and sex-specific scales
1991 Revision	1991	Unified cross-sex syndrome scales; updated norms; 8 core syndromes
2001 Revision (Current)	2001	Added DSM-oriented scales; new normative data; revised items; multi-society compatibility
Multicultural Supplement	2007	Multi-society norms based on data from 31 societies

4.3 Key Differences: 1991 vs. 2001 Editions

Feature	1991 Edition	2001 Edition (Current)
Syndrome Scales	8 empirical syndromes	Same 8 syndromes (refined)
DSM-Oriented Scales	Not included	6 DSM-oriented scales added
Norms	US-based only	US norms + multi-society norms
Item changes	--	Minor item revisions; some items replaced
Computer scoring	Available	Enhanced ASEBA software
Cross-informant comparison	Basic	Integrated multi-informant Q-correlations

4.4 CBCL/1.5-5 (Preschool Version) -- Key Differences from CBCL/6-18

Feature	CBCL/6-18	CBCL/1.5-5
Age range	6-18 years	1.5-5 years
Problem items	113	99
Syndrome scales	8	7 (no Rule-Breaking; combines into one scale)
Competence scales	Activities, Social, School	Not included (Language Development Survey instead)
Broad-band	Internalizing, Externalizing, Total	Same structure
DSM scales	6	5

5. STRUCTURE & CONTENT (CBCL/6-18)

The CBCL/6-18 is organized into two major sections:

5.1 PART I: COMPETENCE ITEMS (20 Items)

These items assess the child's positive functioning and engagement:

I. Activities Scale

- Item types: Sports participated in, hobbies and activities, organizations/clubs, jobs/chores
- Scoring dimensions: Number of activities, amount of time spent, skill level compared to peers
- Example items:
 - "Please list the sports your child most likes to take part in" (up to 3)
 - Rate time spent and skill compared to age-mates: less than average / average / more than average

II. Social Scale

- Item types: Number of close friends, frequency of contact with friends, behavior with siblings and other children, behavior alone
- Scoring dimensions: Quantity of relationships, quality of peer interaction
- Example items:
 - "About how many close friends does your child have?" (none / 1 / 2 or 3 / 4 or more)
 - "How well does your child get along with brothers and sisters?" (worse / average / better)
 - "How well does your child play and work alone?" (worse / average / better)

III. School Scale

- Item types: Academic performance in specific subjects, special class placement, grade repetition, other school problems
- Scoring dimensions: Academic achievement level, presence of learning difficulties
- Example items:
 - Rate performance in each academic subject: Failing / Below Average / Average / Above Average
 - "Is your child in a special class or special school?" (Yes/No -- describe)

- "Has your child repeated a grade?" (Yes/No -- describe)

5.2 PART II: BEHAVIOR PROBLEM ITEMS (113 Items)

The 113 items describe specific behavioral and emotional problems. Each item is rated on a 3-point scale for the past 6 months:

- 0 = Not True (as far as you know)
- 1 = Somewhat or Sometimes True
- 2 = Very True or Often True

Content Domains Covered:

Domain	Sample Items (paraphrased)
Anxiety/Fears	Fears certain animals/situations/places; nervous/tense; too fearful
Depression/Mood	Cries a lot; unhappy/sad/depressed; feels worthless
Withdrawal	Rather be alone; won't talk; secretive
Somatic symptoms	Aches/pains; headaches; nausea; stomach problems; vomiting
Social difficulties	Not liked by peers; gets teased; acts too young
Thought disturbance	Hears sounds/voices not there; sees things not there; strange ideas
Attention/Hyperactivity	Can't concentrate; can't sit still; impulsive; daydreams
Rule-breaking	Lacks guilt; lies/cheats; steals; truancy; runs away
Aggression	Argues; physically attacks; destroys things; threatens; cruelty
Sexual behavior	Thinks about sex too much; sexual problems
Self-harm	Deliberately harms self; talks about killing self
Substance use	Uses alcohol/drugs without medical purpose

Special Item: Item 56 (Physical Problems Without Known Medical Cause)

Item 56 contains sub-items a through h, each describing a specific somatic complaint:

- a. Aches or pains (not stomach or headaches)
- b. Headaches
- c. Nausea, feels sick
- d. Problems with eyes (not if corrected by glasses)
- e. Rashes or other skin problems
- f. Stomachaches
- g. Vomiting, throwing up
- h. Other (specify)

Open-Ended Items

Three items allow parents to describe additional problems:

- Item 113: "Please write in any problems your child has that were not listed above"
- Additional spaces for elaboration on specific items

6. ADMINISTRATION PROCEDURE

6.1 Before Administration

Materials Needed:

- CBCL/6-18 form (official ASEBA form -- copyrighted)
- Pen or pencil
- Quiet, comfortable setting
- Scoring profile (hand-scoring) OR ASEBA computer software

Pre-Administration Checklist:

- ☐ Confirm child's age falls within 6-18 range
- ☐ Identify appropriate informant (parent or parent-surrogate who knows child well)
- ☐ Ensure informant has adequate literacy (approximately 5th-grade reading level)
- ☐ Prepare the setting -- private, minimal distractions
- ☐ Have both parents fill out separate forms if possible

6.2 Instructions to the Parent

Provide the following explanation (adapted from the manual):

"This form asks about your child's behavior and activities. I'd like you to fill it out based on how [child's name] has been over the past 6 months. The checklist covers a wide variety of behaviors th

"For each item, circle 0 if the item is not true of your child, 1 if it is somewhat or sometimes true, and 2 if it is very true or often true. Please answer all items as well as you can."

6.3 Step-by-Step Administration

Step 1: Demographic Information

- Fill in child's full name, sex, age, date of birth, ethnicity/race
- Fill in parent's type of work, date form is filled out
- Record grade in school, name of school

Step 2: Part I -- Competence Section

- Guide parent through the activities, social, and school sections
- Ensure they understand the comparison format ("compared to others of the same age")
- Clarify: "less than average," "average," "more than average"

Step 3: Part II -- Behavior Problems

- Instruct parent to rate each of the 113 items
- Remind them to consider the past 6 months
- Emphasize: answer every item; if uncertain, make best estimate
- For Item 56 (physical problems): rate each sub-item (a-h) separately

Step 4: Review for Completeness

- Check that all items have been answered
- Ask parent to complete any skipped items
- Ensure open-ended items have been addressed if relevant

6.4 Special Administration Situations

Situation	Procedure
Parent with limited literacy	Administer as structured interview -- read items aloud, record responses
Non-English-speaking parent	Use official translated version (available in 100+ languages from ASEBA)
Divorced/separated parents	Each parent fills out separately -- compare profiles
Foster/adoptive parent	Can complete if has known child >=6 months; note informant status
Guardian/relative	Acceptable if knows child well; note relationship
Parent questions item meaning	Clarify factually without influencing response direction
Parent refuses specific items	Note as missing; do not force. Document refusal.

6.5 Important Administration Rules

1. Do NOT paraphrase items -- read exactly as written if administering orally
2. Do NOT provide examples of what counts as "sometimes" vs. "often" -- let parent decide
3. Do NOT discuss items with the child present
4. DO encourage completion of all items -- remind that many items won't apply
5. DO note if parent appears to minimize or exaggerate (clinical observation)
6. DO record time taken and any difficulties observed during completion

6.6 Timing

- Self-administered: Typically 15-20 minutes
- Interview-administered: 20-30 minutes
- Allow extra time for parents who are slow readers or have questions

7. SCORING SYSTEM (Based on Manual -- Appendix A: Instructions for Hand Scoring the

7.1 Overview of Scoring

The CBCL uses a hierarchical scoring system:

Individual Items (0, 1, 2)

IMPORTANT (from manual): Be certain to use a scoring Profile appropriate for the child's sex and age. If a parent checks more than 1 box where only 1 should be checked, score the average of the 2 boxes checked.

7.2 PART I: SCORING THE SOCIAL COMPETENCE SCALES (Exact Manual Procedure)

A. ACTIVITIES SCALE

Do NOT score this scale if data are missing for more than 1 of the 6 scores below (I-A, I-B, II-A, II-B, IV-A, IV-B).

I-A. Number of Sports

Parent Reports	Score Entered on Profile
0 or 1 sport	Enter 0
2 sports	Enter 1
3 or more sports	Enter 2

I-B. Mean Score for Participation & Skill in Sports

- If parent reports no sports -> enter 0
- For EACH sport listed, rate both "time spent" and "skill level":
 - Less Than Average / Below Average = 0
 - Average = 1
 - More Than Average / Above Average = 2
- Exclude blanks and "Don't Know" responses
- Compute the MEAN = (sum of all participation + skill scores) / (number of scores summed)
- Enter this mean on the Profile

II-A. Number of Nonsports Activities

Do NOT count listening to radio, TV, or "goofing off" as activities.

Parent Reports	Score Entered on Profile
0 or 1 activity	Enter 0
2 activities	Enter 1
3 or more activities	Enter 2

II-B. Mean of Participation & Skill in Activities

- Compute mean exactly as specified for sports in I-B above
- Same scoring: Less Than Average = 0, Average = 1, More Than Average = 2
- Exclude blanks and "Don't Know"
- Enter mean on Profile

IV-A. Number of Jobs or Chores

Include both paying and nonpaying jobs and chores.

Parent Reports	Score Entered on Profile
0 or 1 job	Enter 0
2 jobs	Enter 1
3 or more jobs	Enter 2

IV-B. Mean Job Quality

- For each job: Below Average = 0, Average = 1, Above Average = 2
- Exclude blanks and "Don't Know"
- Compute mean = sum of scores / number of scores
- Enter mean on Profile

TOTAL SCORE FOR ACTIVITIES SCALE

- Sum the 6 scores just entered (I-A + I-B + II-A + II-B + IV-A + IV-B)
- Missing data rule: If missing data prevent computation of 1 score, substitute the mean of the other 5 for the missing score
- After computing the total, round off to the nearest .5
- Mark the corresponding number in the column appropriate for the child's age

B. SOCIAL SCALE

Do NOT score this scale if data are missing for more than 1 of the 6 scores below (III-A, III-B, V-1, V-2, VI-A, VI-B).

III-A. Number of Organizations

| Parent Reports | Score Entered on Profile |
| 0 or 1 organization | Enter 0 |

| 2 organizations | Enter 1 |
| 3 or more organizations | Enter 2 |

III-B. Mean of Participation in Organizations

- For each organization: Less Active = 0, Average = 1, More Active = 2
- Exclude "Don't Know"
- Compute mean = sum / number of scores
- Enter mean on Profile

V-1. Number of Friends

Parent Reports	Score Entered on Profile
0 or 1 friend	Enter 0
2 or 3 friends	Enter 1
4 or more friends	Enter 2

V-2. Contacts with Friends

- If parent checks "None" in V-1 -> enter 0

Parent Reports	Score Entered on Profile
Less than 1 time/week	Enter 0
1 or 2 times/week	Enter 1
3 or more times/week	Enter 2

VI-A. Behavior with Others (Items a, b, c)

For each of the three questions (gets along with siblings, with other children, behaves with parents):

- Worse = 0
- About the same = 1
- Better = 2

Excluding any item for which the parent did not check a box, compute the MEAN of the scores and enter on Profile:

VI-B. Play/Work by Self (Item d)

- Worse = 0 -> enter 0
- About the same = 1 -> enter 1
- Better = 2 -> enter 2

TOTAL SCORE FOR SOCIAL SCALE

- Sum the 6 scores (III-A + III-B + V-1 + V-2 + VI-A + VI-B)
- Missing data rule: If missing data prevent computation of 1 score, substitute the mean of the other 5
- Round off to the nearest .5
- Mark the corresponding number in the column appropriate for the child's age

C. SCHOOL SCALE

Do NOT score if: the child is below age 6, not in school, or if data are missing for ANY of the 4 scores (VII-1 through VII-4).

VII-1. Mean Academic Performance

For each academic subject checked:

- Failing = 0
- Below Average = 1
- Average = 2
- Above Average = 3

Enter the MEAN of these scores on the Profile.

Academic subjects INCLUDE: Reading, Writing, Arithmetic/Math, Spelling, Science, English, Foreign Language, History, Social Studies, and similar subjects.

Do NOT count: Physical Education, Art, Music, Home Economics, Driver Education, Industrial Arts, Typing, or similar non-academic subjects.

VII-1 = sum of academic scores / number of subjects rated

VII-2. Special Class

- In ANY type of remedial special class (retarded, emotionally disturbed, learning disabled, perceptual-motor handicapped, reading readiness, resource room, etc.) -> enter 0
- NOT in remedial class -> enter 1

VII-3. Repeated Grade

- Any grades were repeated -> enter 0
- No grades repeated -> enter 1

VII-4. School Problems

- If any school problem is stated that was present in the last 6 months and was NOT already scored above -> enter 0
- No problem beside those scored above -> enter 1

TOTAL SCORE FOR SCHOOL SCALE

- Sum the 4 scores (VII-1 + VII-2 + VII-3 + VII-4)
- Do NOT score if 1 or more scores is missing
- Round off to the nearest .5
- Mark the corresponding number in the age-appropriate column

D. TOTAL SOCIAL COMPETENCE SCORE (Optional)

- Sum the totals of the 3 scales (Activities + Social + School)
- Convert to T-score using the tables in the manual (age- and sex-specific)

7.3 PART II: SCORING THE BEHAVIOR PROBLEM SCALES (Exact Manual Procedure)

Do NOT score if data are missing for more than 8 items (not counting #56h and #113), unless it is clear that the parent intended blanks to be 0s.

If a parent circles two numbers for an item -> score the item 1.

Step 1: Score Individual Items

Place the appropriate template (Page 3 template for child's age and sex) over Page 3 of the CBCL. The Roman numerals and letters beside each item number indicate the scales on which the item is scored. If the parent circled 1 or 2 beside an item, ent

Step 2: Apply Scoring Rules for Described Items

Comments written by the parent MUST be used in judging whether items deserve to be scored, with the following guidelines (from manual):

- | Rule | Explanation |
- | (a) Most specific item rule | For each behavior reported, ONLY the CBCL item that most specifically describes the behavior should
- | (b) Extreme behaviors rule | For extreme behaviors (e.g., sets fires, attempts suicide) -- if parent notes that it happened once
- | (c) "Used to do this" rule | If parent notes "used to do this" -> score as the parent scored it, unless it clearly occurred earl
- | (d) When in doubt | Score the item the way the parent scored it, unless it is CLEAR that the parent has scored it inappropriatel

Step 3: Specific Item Scoring Rules (from Manual)

- | Item | Rule |
- | Item 9 (Obsessions) | Do NOT score anything that is clearly not obsessional; e.g., do not score "won't take no for an answer" |
- | Item 28 (Eats nonfood) | Do NOT count sweets and junk food |
- | Item 40 (Hears things) & Item 70 (Sees things) | Do NOT score anxiousness about sounds/sights that others also notice (e.g., afr
- | Item 46 (Nervous movements) | If "can't sit still" or anything entirely covered by Item 10 is entered here -> score ONLY Item 10
- | Item 56d (Problems with eyes) | Do NOT score "wears glasses," "near-sighted," or other ordinary visual problems with an organic
- | Item 66 (Compulsions) | Do NOT score noncompulsive behavior, e.g., "keeps hitting brother" |
- | Item 72 (Sets fires) | Score "playing with matches or lighter" if parent reports it |
- | Item 77 (Sleeps more) | Do NOT score "wants to stay in bed," BUT DO score difficulties in waking the child |
- | Item 83 (Stores up things) | Do NOT score hobby collections such as stamps, dolls, or miniatures |
- | Item 84 (Strange behavior) & Item 85 (Strange ideas) | If what the parent describes is specifically covered by another item -> s
- | Item 105 (Alcohol or drugs) | Do NOT score tobacco or medication |
- | Item 113 (Additional problems) | Score ONLY if not specifically covered by another item. If parent lists more than 1 "other" ite

Step 4: Compute Scale Scores

To obtain the total raw score for each behavior problem scale:

- Sum the 1s and 2s entered for each scale
- Mark the corresponding number in the graphic display above the scale
- Connect the marked numbers to form a profile
- Percentiles (based on normal children) can be read from the LEFT side of the graphic display
- T-scores can be read from the RIGHT side

Step 5: Compute Internalizing and Externalizing Scores

Using the box to the right of the Behavior Problems Profile:

1. Enter the score (0, 1, or 2) for each behavior problem next to the item's number under the heading "Item"
2. Sum the scores to get the Total Internalizing score
3. Likewise sum for the Total Externalizing score

4. Under the heading "Total," locate the total you have obtained
5. The score to the right of that number is the T-score

CRITICAL NOTE (from manual): The Internalizing and Externalizing scores CANNOT be computed by simply adding scale totals, because some items appear on more than one scale. You must use the item-level scoring method.

Step 6: Compute Total Behavior Problem Score

To compute the total behavior problem score:

1. Sum ALL the 1s and 2s on the entire CBCL
2. Enter the sum to the right of the Profile
3. If the parent has entered a problem for Item 56h or 113 that is NOT covered by another item -> include the score for 56h or 113
4. If more than one problem has been entered for Item 113 -> count ONLY the one having the highest score
5. Cross-check: Subtract the number of items scored as present from the sum of 1s and 2s. The difference should equal the number of 2s.
6. Note: The number and sum of items CANNOT be computed by adding scale totals, because some items appear on more than one scale.

Maximum possible Total Behavior Problem Score: If a 2 is scored for all 118 items + 56h + 113 = 240

7.4 T-SCORE CONVERSION TABLES (from Manual -- Appendix A)

T-scores are normalized standard scores (Mean = 50, SD = 10). The manual provides separate conversion tables for:

Norm Group	Pages in Manual
Boys Aged 4-5	Social Competence + Behavior Problems
Boys Aged 6-11	Social Competence + Behavior Problems
Boys Aged 12-16	Social Competence + Behavior Problems
Girls Aged 4-5	Social Competence + Behavior Problems
Girls Aged 6-11	Social Competence + Behavior Problems
Girls Aged 12-16	Social Competence + Behavior Problems

How to use the tables:

1. Find your child's age-sex group
2. Under "Social Competence" -> locate the raw score -> read T-score
3. Under "Behavior Problems" -> locate the raw score -> read T-score
4. The asterisk (*) marks the lower limit of the "normal" range for competence
5. The ">" symbol marks the upper limit of the "normal" range for competence

Cutoff points from the manual (Chapter 7):

- Total Behavior Problem Score at the 90th percentile of the normative group = good cutoff for discriminating clinically-referred from non-referred children
- Combined behavior problem + social competence cutoffs provide the best discrimination

7.5 Handling Missing Data (Complete Rules from Manual)

Situation	Rule
Activities/Social Scale: 1 item missing	Substitute the mean of the other 5 items for the missing score
Activities/Social Scale: >1 item missing	Do NOT score that scale
School Scale: ANY item missing	Do NOT score the School scale
Behavior Problems: 1-8 items missing (excluding 56h and 113)	Acceptable to score; blanks treated as 0 if clearly intended
Behavior Problems: >8 items missing	Do NOT compute behavior problem scale scores or total scores, UNLESS it is clear the respo
Parent circles two numbers for one item	Score the item 1
Same behavior scored on two items	Score ONLY the most specific item
Item 113: More than 1 additional problem listed	Count ONLY the one with the highest score toward total
Item 56h: Other physical problem listed	Include in total if not covered by another item

7.6 Identifying Suspicious Protocols (from Manual)

The manual provides guidelines for identifying protocols that may reflect invalidity:

Suspiciously LOW Total Behavior Problem Scores (suggest denial/non-understanding):

Age Group	Scores suggesting invalidity	Scores suggesting possible denial
4-5 year-olds	0-5	6-15

| 6-11 year-olds | 0-4 | 5-11 |
| 12-16 year-olds | 0-2 | 3-7 |

Suspiciously HIGH Total Behavior Problem Scores (suggest exaggeration/misunderstanding):

Age Group	Suspicious High Score
Boys 4-5	> 129
Boys 6-11	> 121
Boys 12-16	> 114
Girls 4-5	> 108
Girls 6-11	> 127
Girls 12-16	> 120

Total Social Competence Scores:

- T-score < 13 -> suggests unusually negative picture
- T-score > 78 -> suggests unusually favorable picture

7.7 Computer Scoring (ASEBA Software)

The ASEBA Assessment Data Manager (ADM) software:

- Automatically scores all scales
- Generates T-scores, percentile ranks, and profiles
- Performs cross-informant comparisons
- Computes intraclass correlations with Profile types
- Requires 132-column printer for profile output

Note for Indian Context: Hand-scoring profiles are commonly used in Indian clinical settings. Computer scoring requires purchase of ASEBA software licenses from <https://aseba.org>.

8. SYNDROME SCALES (NARROW-BAND) -- 8 Empirically-Based Scales

These eight scales were derived through factor analysis of large samples of referred and non-referred children. They represent empirically derived clusters of co-occurring problems.

Scale I: Anxious/Depressed

Feature	Details
Core construct	Anxiety, fearfulness, sadness, self-consciousness, feelings of worthlessness
Number of items	13 items
Key items (paraphrased)	Cries a lot; fears school; fears doing bad things; must be perfect; feels unloved; feels worthless; ne
Clinical meaning	Elevated scores suggest subjective emotional distress combining anxious arousal with depressive cognitions. 0
Differential note	Distinguished from Withdrawn/Depressed by the presence of active anxiety symptoms (nervousness, fears) along

Scale II: Withdrawn/Depressed

Feature	Details
Core construct	Social withdrawal, lack of energy, anhedonia, preference for solitude
Number of items	8 items
Key items (paraphrased)	Enjoys little; prefers being alone; refuses to talk; secretive; underactive; unhappy/sad; withdrawn; 1
Clinical meaning	Reflects behavioral manifestations of depression -- the child shows reduced engagement with the social world,
Differential note	Distinguished from Anxious/Depressed by emphasis on behavioral withdrawal and anhedonia rather than active a

Scale III: Somatic Complaints

Feature	Details
Core construct	Physical symptoms without known medical cause
Number of items	11 items
Key items (paraphrased)	Feels dizzy; overtired; aches and pains; headaches; nausea; eye problems; skin problems; stomachaches;
Clinical meaning	Elevated scores suggest somatization -- the expression of emotional distress through physical symptoms. Rule
Clinical caution	Always recommend medical evaluation to rule out organic causes before attributing to psychological factors

Scale IV: Social Problems

| Feature | Details |

| Core construct | Difficulties in peer relationships, social immaturity, social rejection |
 | Number of items | 11 items |
 | Key items (paraphrased) | Acts too young; clings to adults; doesn't get along with peers; gets teased; not liked; clumsy; prefer
 | Clinical meaning | Reflects social skills deficits, peer rejection, and social immaturity. May indicate developmental delays, au
 | Classification | "Neither" scale -- not included in Internalizing or Externalizing broad-band groupings |

Scale V: Thought Problems

Feature	Details
Core construct	Bizarre thoughts, strange behaviors, hallucination-like experiences, obsessions
Number of items	15 items
Key items (paraphrased)	Can't get mind off certain thoughts; hears things not there; sees things not there; does things other
Clinical meaning	Elevated scores may suggest thought disturbance ranging from obsessive-compulsive features to more severe per
Classification	"Neither" scale
Critical threshold	Scores >=70 should prompt thorough evaluation for thought disorder

Scale VI: Attention Problems

Feature	Details
Core construct	Inattention, hyperactivity, impulsivity, disorganization
Number of items	10 items
Key items (paraphrased)	Can't concentrate; can't sit still; confused; daydreams; impulsive; poor school work; inattentive; ner
Clinical meaning	Elevated scores suggest difficulties with sustained attention, behavioral regulation, and impulse control con
Classification	"Neither" scale
Clinical note	Must be present across settings (home + school) for ADHD diagnosis -- compare CBCL with TRF

Scale VII: Rule-Breaking Behavior

Feature	Details
Core construct	Norm-violating behaviors, delinquent acts, dishonesty, substance use
Number of items	17 items
Key items (paraphrased)	Drinks alcohol; lacks guilt; breaks rules; bad companions; lying; prefers older children; runs away; s
Clinical meaning	Elevated scores indicate a pattern of antisocial behavior involving violation of social norms, rules, and the
Classification	Externalizing scale
Developmental note	Some items (substance use, truancy) are more relevant for adolescents than younger children

Scale VIII: Aggressive Behavior

Feature	Details
Core construct	Hostile, oppositional, and physically aggressive behavior; emotional dysregulation in interpersonal contexts
Number of items	18 items
Key items (paraphrased)	Argues; mean to others; demands attention; destroys own things; destroys others' things; disobedient a
Clinical meaning	Elevated scores indicate overtly hostile, confrontational, and physically aggressive behavior. Distinguished
Classification	Externalizing scale

8.1 Summary Table: All 8 Syndrome Scales

#	Scale	Items	Classification	Core Theme
I	Anxious/Depressed	13	Internalizing	Anxiety + depressive cognitions
II	Withdrawn/Depressed	8	Internalizing	Social withdrawal + anhedonia
III	Somatic Complaints	11	Internalizing	Physical symptoms without medical cause
IV	Social Problems	11	Neither	Peer difficulties + social immaturity
V	Thought Problems	15	Neither	Bizarre thoughts + perceptual disturbance
VI	Attention Problems	10	Neither	Inattention + hyperactivity-impulsivity
VII	Rule-Breaking Behavior	17	Externalizing	Norm violations + antisocial acts
VIII	Aggressive Behavior	18	Externalizing	Hostility + physical aggression

9. BROAD-BAND SCALES (Internalizing / Externalizing / Total Problems)

9.1 Conceptual Framework

The broad-band scales represent a higher-order grouping of the eight syndrome scales into two overarching dimensions of psychopathology, plus an overall severity index:

INTERNALIZING (Over-controlled problems)

EXTERNALIZING (Under-controlled problems)

NEITHER (Mixed -- not assigned to either broadband)

TOTAL PROBLEMS = All 113 items (regardless of scale assignment)

9.2 Internalizing Scale

Feature	Details
Definition	Problems that are primarily within the self -- emotions directed inward
Component scales	Anxious/Depressed + Withdrawn/Depressed + Somatic Complaints
Scoring	Sum of items on the three internalizing syndrome scales (items appearing on multiple scales counted only once)
Clinical meaning	Elevated Internalizing reflects an "over-controlled" pattern where the child experiences distress internally
Associated presentations	Depression, anxiety disorders, somatic symptom disorders, school refusal, selective mutism
Behavioral presentation	Quiet, withdrawn, tearful, clingy, complaining of physical symptoms, avoiding activities

9.3 Externalizing Scale

Feature	Details
Definition	Problems that are directed outward toward the environment and other people
Component scales	Rule-Breaking Behavior + Aggressive Behavior
Scoring	Sum of items on the two externalizing syndrome scales (items appearing on both counted only once)
Clinical meaning	Elevated Externalizing reflects an "under-controlled" pattern where the child expresses distress through conf
Associated presentations	ADHD, ODD, Conduct Disorder, intermittent explosive disorder, substance use
Behavioral presentation	Argues, fights, defies, lies, steals, destroys, disrupts, is impulsive

9.4 Total Problems Score

Feature	Details
Definition	Overall severity of behavioral and emotional problems
Scoring	Sum of ALL 113 behavior problem items (including items not assigned to any syndrome scale)
Clinical meaning	Provides a global index of overall problem severity. Useful as a general screening indicator and for tracking
Note	Approximately 17 items contribute to Total Problems but are not assigned to any of the 8 syndrome scales

9.5 The "Neither" Scales

Social Problems, Thought Problems, and Attention Problems are categorized as "neither internalizing nor externalizing" because:

- They did not load clearly on either the Internalizing or Externalizing factor in factor-analytic studies
- They represent "mixed" problems with both internal and external features
- Attention Problems, for example, have both an internal component (can't concentrate) and external manifestations (hyperactivity)
- They still contribute to the Total Problems score

9.6 Clinical Profiles Based on Broad-Band Patterns

Pattern	Interpretation	Common Presentations
Internalizing > Externalizing	Over-controlled; distress turned inward	Depression, anxiety, psychosomatic disorders
Externalizing > Internalizing	Under-controlled; distress turned outward	ODD, Conduct Disorder, ADHD
Both Elevated	Mixed/complex presentation	Comorbid conditions; complex trauma; severe disturbance
Neither Elevated	Within normal limits OR problems concentrated on "neither" scales	May still have specific elevations on Soc
Total Problems elevated but individual scales normal	Diffuse, non-specific problems	Subthreshold across multiple domains; ge

10. DSM-ORIENTED SCALES

10.1 Background

The DSM-Oriented Scales were added in the 2001 revision to bridge the gap between the empirically-derived syndrome scales and the clinically-used DSM diagnostic categories. They were developed through a unique method:

Development Process:

1. Panels of psychiatrists and psychologists from multiple countries reviewed each CBCL item
2. Experts rated whether each item was "very consistent," "somewhat consistent," or "not consistent" with specific DSM-IV diagnostic categories
3. Items rated as "very consistent" by a majority ($\geq 64\%$) of experts were assigned to the corresponding DSM-oriented scale

Important: These scales are NOT diagnostic instruments -- they indicate the degree to which a child's profile is consistent with DSM categories, providing hypotheses for further evaluation.

10.2 The Six DSM-Oriented Scales

1. Affective Problems

Feature	Details
DSM Correspondence	Major Depressive Disorder / Dysthymia
Number of items	13 items
Core content	Sadness, loss of interest, sleep disturbance, fatigue, worthlessness, guilt, appetite change, suicidal ideation
Key items (paraphrased)	Cries a lot; sleep problems; feels worthless; lacks energy; little interest; appetite changes; unhappy
Clinical note	Elevated scores warrant mood disorder evaluation; inquire about duration, functional impairment, and vegetative

2. Anxiety Problems

Feature	Details
DSM Correspondence	Generalized Anxiety Disorder, Separation Anxiety Disorder, Specific Phobia
Number of items	6 items
Core content	Excessive worry, separation fears, specific fears, nervousness, physical anxiety symptoms
Key items (paraphrased)	Fears animals/situations/school; too nervous/tense; fears; too fearful; worries
Clinical note	Brief scale; elevated scores suggest anxiety but do not differentiate between specific anxiety diagnoses -- furt

3. Somatic Problems

Feature	Details
DSM Correspondence	Somatization Disorder / Somatic Symptom Disorder
Number of items	7 items
Core content	Multiple physical complaints without known medical cause
Key items (paraphrased)	Aches; headaches; nausea; eye problems; skin problems; stomachaches; vomiting
Clinical note	Medical evaluation should be completed before interpreting elevated scores as psychosomatic; consider cultural i

4. Attention Deficit/Hyperactivity Problems (ADHD Problems)

Feature	Details
DSM Correspondence	Attention-Deficit/Hyperactivity Disorder
Number of items	7 items
Core content	Inattention, hyperactivity, impulsivity, disorganization
Key items (paraphrased)	Can't concentrate; can't sit still; impulsive; fails to finish; inattentive; fidgets; confused
Clinical note	ADHD diagnosis requires cross-setting impairment -- MUST compare with TRF scores; onset before age 12; duration

5. Oppositional Defiant Problems

Feature	Details
DSM Correspondence	Oppositional Defiant Disorder (ODD)
Number of items	5 items
Core content	Defiance, argumentativeness, anger, irritability
Key items (paraphrased)	Argues; disobedient at home; stubborn; temper tantrums; defiant
Clinical note	Distinguish from normative oppositional behavior at certain developmental stages (e.g., early adolescence); cons

6. Conduct Problems

Feature	Details
DSM Correspondence	Conduct Disorder (CD)
Number of items	15 items
Core content	Serious norm violations, aggression toward people/animals, destruction, deceitfulness, theft
Key items (paraphrased)	Cruel to animals; fighting; attacks; runs away; sets fires; steals; threatens; truancy; vandalism; lie
Clinical note	Elevated scores indicate serious antisocial behavior -- warranting comprehensive evaluation; consider age of onset

10.3 Summary Table: DSM-Oriented Scales

DSM Scale	DSM Category	Items	Key Clinical Question
Affective Problems	MDD / Dysthymia	13	Is this child experiencing a depressive episode?
Anxiety Problems	GAD / SAD / Phobia	6	Does this child have clinically significant anxiety?
Somatic Problems	Somatic Symptom Disorder	7	Is this child somatizing emotional distress?
ADHD Problems	ADHD	7	Is this child showing ADHD symptoms?
Oppositional Defiant Problems	ODD	5	Is this child showing oppositional patterns?
Conduct Problems	Conduct Disorder	15	Is this child showing antisocial behavior patterns?

10.4 Relationship Between Syndrome Scales and DSM Scales

The empirically-derived syndrome scales and DSM-oriented scales overlap but are not identical:

- Anxious/Depressed syndrome scale partially overlaps with both Affective Problems and Anxiety Problems DSM scales
- Rule-Breaking and Aggressive Behavior overlap with both ODD and Conduct Problems DSM scales
- Somatic Complaints largely overlaps with Somatic Problems DSM scale

- Some items appear on a syndrome scale but NOT on any DSM scale, and vice versa

Clinical Implication: Report BOTH syndrome scales and DSM-oriented scales -- they provide complementary information.

11. COMPETENCE SCALES

11.1 Overview

The Competence Scales assess the child's positive functioning -- strengths, activities, social engagement, and academic performance. They are unique to the CBCL (not available on TRF or YSR in the same form) and provide essential information about ad

CRITICAL INTERPRETATION RULE: For competence scales, LOWER T-scores indicate MORE problems (the opposite direction from behavior problem scales).

11.2 Activities Scale

Feature	Details
What it measures	Breadth and quality of involvement in sports, hobbies, organizations, and jobs/chores
Scoring components	Number of activities + time spent + skill level
Example	A child who plays 2 sports, average time, above-average skill, has 3 hobbies, is in 2 organizations, and does 1 chore
Low score meaning	Child participates in few activities, spends less time, and/or performs at below-average skill level
Clinical implication	Low Activities scores may reflect: social withdrawal, depression, restricted interests (ASD), physical li

11.3 Social Scale

Feature	Details
What it measures	Quantity and quality of social relationships and interpersonal behavior
Scoring components	Number of friends + frequency of contact + quality of interaction with peers, siblings, and parents
Example	A child with 4+ friends, sees them 2+ times/week, gets along better than average with siblings and peers = high social
Low score meaning	Few friends, infrequent social contact, poor relationships with peers/siblings
Clinical implication	Low Social scores suggest social skills deficits, social anxiety, peer rejection, ASD features, or conseq

11.4 School Scale

| Feature | Details |
| What it measures | Academic performance and school functioning |
| Scoring components | Average performance across subjects, minus penalties for special class placement, grade retention, or schoo
| Example | A child with average grades in all subjects, no special services, no grade retention = average school score |
| Low score meaning | Below-average academic performance, special education placement, grade retention, or other school problems |
| Clinical implication | Low School scores may indicate: learning disabilities, intellectual limitations, ADHD impact on academics

11.5 Total Competence Score

Feature	Details
What it measures	Overall adaptive functioning across activities, social, and academic domains
Calculation	Sum of Activities + Social + School raw scores -> convert to T-score
Clinical meaning	Provides a global index of the child's positive functioning and engagement with the world
Interpretive rule	A child can have elevated problem scores AND low competence scores (dual deficit); OR elevated problems with

11.6 Competence Scale Cutoffs (REVERSED Direction)

REMEMBER: For competence, LOW scores = PROBLEMS

Scale	Normal	Borderline	Clinical
Activities	T > 35	T = 31-35	T < 31
Social	T > 35	T = 31-35	T < 31
School	T > 35	T = 31-35	T < 31
Total Competence	T > 37	T = 33-37	T < 33

12. T-SCORE INTERPRETATION & CLINICAL CUTOFFS

12.1 Understanding T-Scores

T-scores are normalized standard scores that allow comparison to a normative sample:

	T-Score		Meaning	
	50		Exactly average (mean of normative sample)	
	60		1 SD above mean (more problems than ~84% of norm sample)	
	65		1.5 SD above mean (more problems than ~93% of norm sample)	
	70		2 SD above mean (more problems than ~97% of norm sample)	
	40		1 SD below mean (fewer problems than average)	

12.2 Clinical Cutoffs for SYNDROME SCALES (Narrow-Band)

	Range		T-Score		Percentile		Clinical Meaning	
	Normal		< 65		< 93rd		Within normal limits; typical for age and sex	
	Borderline Clinical		65-69		93rd-97th		Elevated; at-risk; warrants monitoring and possible intervention	
	Clinical		>= 70		> 97th		Clinically significant; child scores higher than 97% of normative peers; intervention indicated	

12.3 Clinical Cutoffs for BROAD-BAND SCALES (Internalizing, Externalizing, Total Problems)

Note: Cutoffs for broad-band scales are LOWER than for syndrome scales because broad-band scales aggregate more items and have greater reliability.

	Range		T-Score		Percentile		Clinical Meaning	
	Normal		< 60		< 84th		Within normal limits	
	Borderline Clinical		60-63		84th-90th		Elevated; at-risk; warrants monitoring	
	Clinical		>= 64		> 90th		Clinically significant; intervention indicated	

12.4 Clinical Cutoffs for COMPETENCE SCALES (Direction REVERSED)

CRITICAL: For competence scales, LOWER scores = MORE impairment

	Scale		Normal		Borderline Clinical		Clinical	
	Activities		T > 35		T = 31-35		T < 31	
	Social		T > 35		T = 31-35		T < 31	
	School		T > 35		T = 31-35		T < 31	
	Total Competence		T > 37		T = 33-37		T < 33	

12.5 Clinical Cutoffs for DSM-ORIENTED SCALES

The DSM-oriented scales use the same cutoffs as syndrome scales:

	Range		T-Score		Interpretation	
	Normal		< 65		Not consistent with DSM category	
	Borderline Clinical		65-69		Somewhat consistent; warrants further evaluation	
	Clinical		>= 70		Highly consistent with DSM category; comprehensive evaluation indicated	

12.6 Quick Reference Decision Grid

	If the score is...		Then...	
	All scales Normal (T < 65 syndrome; T < 60 broad-band)		No clinical concern on CBCL; consider other data sources	
	Borderline on 1-2 scales		Monitor; consider brief intervention; re-evaluate in 3-6 months	
	Clinical on 1+ syndrome scale		Warrants intervention; further diagnostic evaluation indicated	
	Clinical on broad-band scale		Significant overall pattern; comprehensive treatment planning needed	
	Clinical on Total Problems		Pervasive difficulties; intensive intervention likely needed	
	Competence in Clinical range		Significant functional impairment; strengthen adaptive functioning	

12.7 Important Notes on Cutoffs

1. Cutoffs are NOT diagnostic thresholds -- they indicate the need for further evaluation, not the presence of a disorder
2. Borderline range is clinically important -- do not dismiss scores in the 65-69 range as "normal"
3. Context matters -- a child at T=69 (borderline) is clinically indistinguishable from one at T=70 (clinical); interpretation should not be rigidly categorical
4. Cultural considerations -- cutoffs are based on US norms; multi-society norms (2007) may be more appropriate for some populations
5. Statistical confidence -- small differences in T-scores (e.g., T=65 vs T=67) may not be clinically meaningful given measurement error (SEM ~ 2-4 T-score points)

13. INTERPRETATION GUIDELINES

13.1 General Principles of Interpretation

1. Never interpret the CBCL in isolation -- integrate with clinical interview, observation, history, and other test data
2. Consider the informant -- who filled out the form? What is their relationship to the child? Any biases (custody dispute, overprotective parent, minimizing parent)?
3. Consider developmental context -- some behaviors are normative at certain ages (e.g., tantrums in a 6-year-old vs. a 16-year-old)
4. Consider cultural context -- behavior expectations vary across cultures; some items may carry different meanings
5. Look at patterns, not isolated scores -- a profile with multiple borderline elevations may be more concerning than a single clinical elevation
6. Report strengths alongside problems -- competence data is clinically essential

13.2 Step-by-Step Interpretation Sequence

Follow this order for systematic interpretation:

Step 1: Examine Total Problems Score First

- Is there an overall pattern of elevated difficulty?
- $T \geq 64$ = Clinically significant overall disturbance
- This gives you the "big picture" before examining specific patterns

Step 2: Examine Broad-Band Scales (Internalizing vs. Externalizing)

- Is the profile predominantly Internalizing, Externalizing, Mixed, or Neither?
- Calculate the Internalizing-Externalizing difference:
 - Difference > 10 T-score points = meaningful pattern difference
 - Both elevated = mixed/complex presentation

Step 3: Examine Individual Syndrome Scales

- Which specific scales are elevated (≥ 65)?
- Note the pattern of elevations -- do they form a coherent clinical picture?
- Identify the HIGHEST scale -- this is likely the most prominent feature

Step 4: Examine DSM-Oriented Scales

- Do DSM-oriented scale elevations support diagnostic hypotheses?
- Are they consistent with the empirically-derived syndrome scale elevations?

Step 5: Examine Competence Scales

- Is competence adequate despite problems (protective factor)?
- Is competence also impaired (compound vulnerability)?
- Which domain of competence is most affected?

Step 6: Consider Critical Items

- Items related to self-harm (Item 18, 91)
- Items related to hallucinations (Items 40, 70)
- Items related to fire-setting (Item 72)
- Items related to cruelty (Items 15, 21)
- Items related to sexual problems (Items 59, 60, 73)
- ANY item scored "2" on a critical item warrants immediate clinical follow-up

Step 7: Cross-Informant Comparison (if available)

- Compare CBCL (parent) with TRF (teacher) and YSR (self)
- Agreement suggests pervasive problems
- Discrepancy suggests context-specific problems or informant bias

13.3 Common Profile Patterns and Their Clinical Meaning

Pattern 1: Pure Internalizing Profile

Elevated: Anxious/Depressed, Withdrawn/Depressed, Somatic Complaints

Pattern 2: Pure Externalizing Profile

Elevated: Rule-Breaking, Aggressive Behavior

Pattern 3: Mixed Internalizing + Externalizing

Elevated: Both Internalizing AND Externalizing broad-band scales

Pattern 4: Attention Problems + Externalizing

Elevated: Attention Problems, Rule-Breaking, Aggressive Behavior

Pattern 5: Thought Problems + Social Problems

Elevated: Thought Problems, Social Problems ± Withdrawn/Depressed

Pattern 6: Somatic Complaints Isolated Elevation

Elevated: Somatic Complaints only (other scales normal)

13.4 Interpreting Cross-Informant Discrepancies

| Pattern | Meaning | Clinical Implication |
| Parent elevated, Teacher normal | Problems may be home-specific | Explore family dynamics, parenting stress, parental psychopath
| Teacher elevated, Parent normal | Problems may be school-specific OR parent is minimizing | Explore academic demands, peer relat
| Both elevated | Pervasive, cross-situational problems | Likely meets criteria for clinical disorder; more severe |
| Youth self-report elevated, Parent normal | Parent unaware of subjective distress | Common for internalizing problems; take you
| Parent elevated, Youth normal | Possible minimization by youth OR parent over-reporting | Consider developmental stage (adolesce

13.5 Developmental Considerations

Age Group	Interpretive Considerations
6-8 years	Some oppositional behavior normative; attention spans still developing; somatic complaints common with stress
9-11 years	Peer problems become more salient; academic demands increase; rule-breaking items less applicable
12-14 years	Normative increase in mood variability; rule-breaking items more applicable; peer influence increases
15-18 years	Substance use items relevant; independence conflicts normal; distinguish from pathology

14. REPORT WRITING FORMAT

14.1 Standard Report Structure

A CBCL clinical report should follow this format:

I. IDENTIFYING INFORMATION

14.2 Section-by-Section Writing Guide

I. Identifying Information

Include: Name, age, sex, date of birth, grade/class, school, date of assessment, referral source, clinician name and qualifications.

II. Reason for Referral

Brief statement of why the child was referred and what questions the assessment aims to answer.

III. Test Administered & Rationale

State: "The Child Behavior Checklist for ages 6-18 (CBCL/6-18; Achenbach & Rescorla, 2001) was administered to [informant] to obtain a standardized assessment of [child's name]'s behavioral and emotional functioning as perceived by the parent."

IV. Informant Details

State who filled out the form, their relationship to the child, how long they have known the child, and any factors that might affect their report (e.g., custody dispute, limited contact).

V. Behavioral Observations

If the form was administered as an interview: note the parent's engagement, any hesitation, any notable responses, emotional state during completion.

VI. Results

Writing the Results Section -- General Rules:

1. Present data in tables for clarity
2. State T-scores, percentile ranks, and clinical range for each scale
3. Use professional language: "falls within the clinical range" not "is abnormal"
4. Report ALL scales, including normal ones (document that you assessed the full profile)
5. Highlight elevations clearly
6. Use the child's name or "the child" -- not "the patient" (in pediatric reports)

Format for presenting individual scales:

"[Scale Name]: T-score = [XX] (Percentile = [XX]th; [Normal/Borderline Clinical/Clinical Range]). This score indicates that [child's name] was rated by [informant] as showing [more/fewer/similar] prob

VII. Interpretation & Clinical Significance

- Integrate findings into a coherent narrative
- Identify the dominant pattern (Internalizing/Externalizing/Mixed)
- Connect elevations to presenting complaints
- Note consistency/inconsistency with referral concerns
- Discuss developmental and contextual factors

VIII. Cross-Informant Comparison

If multiple forms were completed (e.g., mother and father CBCL, or CBCL + TRF):

- Note areas of agreement
- Note discrepancies and offer possible explanations
- Identify which problems are pervasive vs. context-specific

IX. Summary & Diagnostic Impressions

- Concise paragraph summarizing the overall profile
- State whether findings support, are inconsistent with, or are insufficient to address the referral question
- Generate diagnostic hypotheses (NOT diagnoses) based on the data

X. Recommendations

Based on the CBCL findings:

- Need for further evaluation (specify what kind)
- Recommended interventions
- Re-assessment timeline
- Cross-informant data needs
- Referrals (if indicated)

14.3 Language Guidelines for Report Writing

Instead of...	Write...
"Abnormal score"	"Clinically significant elevation"
"The child is aggressive"	"The parent's report places the child in the clinical range on the Aggressive Behavior scale"
"Diagnosis: ODD"	"The profile is consistent with Oppositional Defiant Disorder and warrants further diagnostic evaluation"
"Normal"	"Within normal limits" or "within the normative range"
"The test shows..."	"The parent's report on the CBCL suggests..."
"Proves"	"Indicates" or "is consistent with"

Key Principle: The CBCL reports the informant's perception -- always attribute findings to the informant, not as absolute facts about the child.

15. SAMPLE REPORT TEMPLATE

CHILD BEHAVIOR CHECKLIST (CBCL/6-18) -- PSYCHOLOGICAL ASSESSMENT REPORT

I. IDENTIFYING INFORMATION

Field	Details
Name	[Child's Full Name]
Age / DOB	[Age] years / [DD-MM-YYYY]
Sex	[Male / Female]
Grade / Class	[Grade]
School	[School Name]
Date of Assessment	[DD-MM-YYYY]
Referred by	[Referral Source]
Informant	[Name] ([Relationship to child])
Clinician	[Name], [Qualifications]

II. REASON FOR REFERRAL

[Child's name] was referred by [referral source] for assessment of [presenting concerns -- e.g., "behavioral difficulties at home and school, including frequent temper outbursts, defiance, and declining academic performance"]. The CBCL was administer

III. TEST ADMINISTERED

Child Behavior Checklist for Ages 6-18 (CBCL/6-18)
- Authors: Achenbach, T.M., & Rescorla, L.A. (2001)
- Informant: [Mother / Father / Guardian]
- Date completed: [Date]
- Validity: The form was completed in full with no missing items. The informant appeared to engage thoughtfully with the items. The results are considered a valid representation of [informant's] percept

IV. RESULTS

A. Competence Scales

Scale	Raw Score	T-Score	Percentile	Range
Activities	[]	[]	[]th	[Normal / Borderline / Clinical]
Social	[]	[]	[]th	[Normal / Borderline / Clinical]
School	[]	[]	[]th	[Normal / Borderline / Clinical]
Total Competence	[]	[]	[]th	[Normal / Borderline / Clinical]

Note: For competence scales, lower T-scores indicate greater impairment.

Narrative: [Child's name]'s overall competence was rated in the [] range (Total Competence T = []). [His/Her] [Activities/Social/School] functioning was rated as [adequate/below expectations/significantly impaired], with [specific details].

B. Syndrome Scales (Narrow-Band)

Syndrome Scale	Raw Score	T-Score	Percentile	Range
I. Anxious/Depressed	[]	[]	[]th	[Normal / Borderline / Clinical]
II. Withdrawn/Depressed	[]	[]	[]th	[Normal / Borderline / Clinical]
III. Somatic Complaints	[]	[]	[]th	[Normal / Borderline / Clinical]
IV. Social Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
V. Thought Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
VI. Attention Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
VII. Rule-Breaking Behavior	[]	[]	[]th	[Normal / Borderline / Clinical]
VIII. Aggressive Behavior	[]	[]	[]th	[Normal / Borderline / Clinical]

C. Broad-Band Scales

Scale	Raw Score	T-Score	Percentile	Range
Internalizing Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Externalizing Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Total Problems	[]	[]	[]th	[Normal / Borderline / Clinical]

D. DSM-Oriented Scales

DSM Scale	Raw Score	T-Score	Percentile	Range
Affective Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Anxiety Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Somatic Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
ADHD Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Oppositional Defiant Problems	[]	[]	[]th	[Normal / Borderline / Clinical]
Conduct Problems	[]	[]	[]th	[Normal / Borderline / Clinical]

E. Critical Items

Critical Item	Score	Clinical Note
Self-harm (Item 18)	[0/1/2]	[Comment if scored 1 or 2]
Suicide talk (Item 91)	[0/1/2]	[Comment if scored 1 or 2]
Hallucinations (Items 40, 70)	[0/1/2]	[Comment if scored 1 or 2]
Fire-setting (Item 72)	[0/1/2]	[Comment if scored 1 or 2]
Cruelty to animals (Item 15)	[0/1/2]	[Comment if scored 1 or 2]
Sexual problems (Items 59, 73)	[0/1/2]	[Comment if scored 1 or 2]

V. INTERPRETATION & CLINICAL SIGNIFICANCE

Overall Profile:

Internalizing vs. Externalizing Pattern:

Specific Elevations:

DSM-Oriented Findings:

Competence:

VI. SUMMARY

[2-3 sentence summary capturing the essential findings:]

[Child's name], a [age]-year-old [boy/girl], was rated by [informant] as showing clinically significant elevations on [list scales]. The overall profile suggests a [predominant pattern] presentation consistent with [clinical hypothesis]. Competence w

VII. DIAGNOSTIC IMPRESSIONS (Hypotheses for Further Evaluation)

Based on the CBCL profile, the following warrant comprehensive diagnostic evaluation:

1. [DSM-5 diagnostic hypothesis #1]
2. [DSM-5 diagnostic hypothesis #2]
3. Rule out: [Differential diagnosis]

Note: The CBCL does not provide formal diagnoses. These impressions are based on the empirical scale pattern and require confirmation through clinical interview, developmental history, and additional assessment.

VIII. RECOMMENDATIONS

1. [Recommendation #1 -- e.g., "Comprehensive clinical interview to assess for Major Depressive Disorder"]
2. [Recommendation #2 -- e.g., "Teacher Report Form (TRF) to assess school-based behavior"]
3. [Recommendation #3 -- e.g., "Individual psychotherapy targeting anxiety and social withdrawal"]
4. [Recommendation #4 -- e.g., "Parent management training for behavioral difficulties"]
5. [Recommendation #5 -- e.g., "Re-administration of CBCL in 6 months to evaluate treatment response"]
6. [Recommendation #6 -- e.g., "Psychoeducational assessment if academic difficulties persist"]

Clinician's Signature: _____

This report is based on information provided by [informant] and should be interpreted in the context of comprehensive clinical assessment. CBCL scores reflect the informant's perception and are not equivalent to clinical diagnoses.

16. PSYCHOMETRIC PROPERTIES

16.1 Reliability

Test-Retest Reliability

| Study | Interval | Scale | Coefficient |
| Achenbach & Rescorla (2001) | 8 days (mean) | Total Problems | $r = .94$ |

Achenbach & Rescorla (2001)	8 days	Internalizing	$r = .91$
Achenbach & Rescorla (2001)	8 days	Externalizing	$r = .92$
Achenbach & Rescorla (2001)	8 days	Syndrome Scales	$r = .82-.94$ (range)
Achenbach & Rescorla (2001)	8 days	Competence Scales	$r = .88-.93$
Achenbach (1991 edition)	7 days	Total Problems	$r = .95$

Interpretation: Excellent short-term stability; scores are highly reproducible over 1-week intervals, indicating the instrument measures relatively stable behavioral patterns rather than momentary states.

Inter-Parent Agreement (Mother-Father)

Scale	Mean Correlation
Problem Scales	$r = .76$ (mean across all scales)
Internalizing	$r = .72$
Externalizing	$r = .81$
Total Problems	$r = .76$
Competence Scales	$r = .72-.85$

Interpretation: Good agreement between mothers and fathers, with somewhat higher agreement on externally observable behaviors (externalizing) than on internal states (internalizing). This is expected -- externalizing behaviors are more easily observe

Internal Consistency (Cronbach's Alpha)

Scale	Alpha (Range across norm groups)
Total Problems	$\alpha = .95-.97$
Internalizing	$\alpha = .90$
Externalizing	$\alpha = .94$
Anxious/Depressed	$\alpha = .84$
Withdrawn/Depressed	$\alpha = .80$
Somatic Complaints	$\alpha = .78$
Social Problems	$\alpha = .82$
Thought Problems	$\alpha = .78$
Attention Problems	$\alpha = .86$
Rule-Breaking Behavior	$\alpha = .85$
Aggressive Behavior	$\alpha = .94$

Interpretation: All scales demonstrate acceptable to excellent internal consistency. Broad-band and Total scores have very high alpha values; individual syndrome scales are somewhat lower but all exceed the .70 threshold for clinical acceptability.

Intraclass Correlation Coefficient (ICC)

- Total Problems: ICC = .97
- Individual syndrome scales: ICC = .93-.97

16.2 Validity

Content Validity

- Items derived from comprehensive review of clinical literature (1960s-2000s)
- Piloted with parents of clinic-referred and non-referred children
- Reviewed by clinicians for relevance and clarity
- Items cover the full range of childhood behavioral/emotional problems recognized in clinical practice
- Cross-cultural content review conducted for translations

Construct Validity

- Factor Structure: Consistent 8-factor structure replicated across:
 - 60+ societies
 - Multiple cultures and languages
 - Both referred and non-referred samples
 - Multiple age groups and both sexes
- Higher-order structure: Internalizing-Externalizing distinction robustly confirmed across cultures
- Convergent validity: CBCL scales correlate significantly with corresponding scales on:
 - Conners' Rating Scales: $r = .71-.85$
 - Behavior Assessment System for Children (BASC): $r = .70-.88$
 - Strengths and Difficulties Questionnaire (SDQ): $r = .72-.87$

Criterion-Related Validity (Discriminative Validity)

| Comparison | Finding |
 | Referred vs. Non-referred | CBCL scores significantly discriminate referred from non-referred children across ALL scales ($p < .0$)
Effect sizes	Large (Cohen's $d = 1.0-2.0$ for Total Problems)
Sensitivity	Approximately 87% of referred children score in borderline or clinical range
Specificity	Approximately 83% of non-referred children score in normal range

Predictive Validity

- CBCL scores at intake predict treatment duration and outcome
- Preschool CBCL scores predict school-age problems (longitudinal studies)
- Elevated scores predict future DSM diagnoses (over 2-14 year follow-ups)
- Total Problems and Externalizing scores at age 4-5 predict adult mental health outcomes

Cross-Cultural Validity

- Translated and validated in 100+ languages
- Used in research across 100+ societies
- Multi-society norms (2007) based on data from 31 societies including:
 - Australia, Belgium, China, Denmark, Ethiopia, France, Germany, Greece, India, Iran, Israel, Italy, Jamaica, Japan, Korea, Netherlands, Norway, Peru, Puerto Rico, Romania, Russia, Sweden, Taiwan, Thailand
- Factor structure remarkably consistent across cultures
- Minor cultural variations in base rates of specific problems (e.g., somatic complaints higher in some Asian cultures)

16.3 Normative Data

Edition	Normative Sample
1983 Original	N = 1,442 non-referred children (matched to 1980 US Census for age, sex, ethnicity, SES)
1991 Revision	N = 2,368 non-referred children (updated US norms)
2001 Revision	N = 1,753 non-referred children (current US norms)
2007 Multicultural	Combined data from 31 societies; culture-specific norms available

Norm Groups (2001 edition):

- Boys ages 6-11
- Boys ages 12-18
- Girls ages 6-11
- Girls ages 12-18

Indian Norms: While no official Indian normative data is published by ASEBA, several Indian studies have validated the CBCL in Indian populations. Clinicians in India typically use the multi-society norms or US norms with cultural interpretive caution

16.4 Standard Error of Measurement (SEM)

Scale Type	Approximate SEM
Syndrome Scales	2-4 T-score points
Broad-Band Scales	2-3 T-score points
Total Problems	2 T-score points

Clinical Implication: A T-score difference of less than 5 points between two administrations may not reflect meaningful clinical change (falls within measurement error). For confident interpretation of change, look for differences of ≥ 5 T-score points

17. ADVANTAGES & LIMITATIONS

17.1 Advantages

#	Advantage	Explanation
1	Extensively validated worldwide	Over 10,000 published studies; translated into 100+ languages; used in 100+ societies
2	Excellent psychometric properties	Consistently high reliability and validity across cultures and populations
3	Empirically-based taxonomy	Scales derived from data (factor analysis) rather than imposed from theory -- captures how problems manifest
4	Quantitative and standardized	T-scores and percentiles allow precise communication, tracking over time, and comparison across settings
5	Multi-informant system	CBCL + TRF + YSR provide comprehensive, cross-setting assessment from multiple perspectives
6	Assesses both problems AND competence	Dual assessment of difficulties and strengths provides complete clinical picture
7	Culturally adaptable	Multi-society norms available; validated across diverse populations
8	Efficient	Only 15-20 minutes to complete; minimal clinician time for administration
9	Bridges empirical and clinical traditions	DSM-oriented scales connect data-driven syndromes to clinical diagnostic categories
10	Tracks treatment response	Repeated administration documents change over time with normative benchmarks
11	Age- and sex-specific norms	Accounts for developmental and sex differences in base rates of behavior
12	Broad coverage	113 items cover the full range of childhood behavioral and emotional problems
13	Both computer and hand-scoring available	Flexible scoring options for different resource settings
14	Minimal training for administration	Any literate adult can fill it out; training needed only for interpretation
15	Strong research base for comparison	Extensive published norms and clinical studies allow meaningful comparison

17.2 Limitations

#	Limitation	Explanation & Mitigation
1	Informant bias	Parent report is subjective -- influenced by parent's own psychopathology, relationship quality, expectations
2	Does not provide diagnoses	Elevated scales suggest possibilities, not confirmed DSM diagnoses. Mitigation: Integrate with clinical interview
3	Requires literacy	Approximately 5th-grade (Class 5) reading level needed. Mitigation: Administer as interview for low-literate
4	Cost barrier	Copyrighted instrument requiring purchase from ASEBA. Cannot be freely reproduced. Mitigation: Institutional purchase
5	Does not assess ALL disorders	PTSD, eating disorders, attachment disorders not directly assessed. Mitigation: Supplement with clinical interview
6	May miss low-frequency critical behaviors	A child who has set fire once (but scored "1") may not appear clinically elevated
7	Cultural item relevance	Some items may carry different meanings across cultures (e.g., "arguments" may be normative vs. pathological)
8	Limited for children under 6	Requires separate preschool form (CBCL/1.5-5) with different norms and scales. Mitigation: Use appropriate form
9	6-month retrospective recall	Parents may not accurately recall behaviors over 6 months; recency bias possible. Mitigation: Use direct observation
10	Cannot replace clinical interview	Structured checklist cannot capture nuance, context, onset, triggers, or functional analysis
11	Potentially outdated items	Some items may not reflect contemporary behavioral concerns (e.g., cyberbullying, internet-related behaviors)
12	Norms primarily Western	Despite multi-society norms, primary standardization is US-based. Mitigation: Interpret with cultural context

17.3 When CBCL Is Particularly Indicated

- Initial intake assessment in child/adolescent mental health services
- Screening for emotional/behavioral problems in pediatric settings
- Baseline and follow-up measurement in treatment outcome research
- Multi-informant assessment (combined with TRF, YSR)
- Epidemiological surveys of child mental health
- Cross-cultural comparison of childhood psychopathology

17.4 When CBCL Is NOT Sufficient Alone

- Formal DSM-5 diagnosis (requires clinical interview)
- Assessment of specific disorders: PTSD, eating disorders, ASD (use specific tools)
- Intellectual/cognitive assessment (use IQ tests)
- Assessment of learning disabilities (use achievement tests)
- Functional behavioral analysis (use direct observation, FBA protocols)
- Crisis assessment (use suicide risk assessment tools)

18. REFERENCES

18.1 Primary References

1. Achenbach, T.M., & Edelbrock, C. (1983). Manual for the Child Behavior Checklist and Revised Child Behavior Profile. Burlington, VT: University of Vermont, Department of Psychiatry.
2. Achenbach, T.M. (1991). Manual for the Child Behavior Checklist/4-18 and 1991 Profile. Burlington, VT: University of Vermont, Department of Psychiatry.
3. Achenbach, T.M., & Rescorla, L.A. (2001). Manual for the ASEBA School-Age Forms & Profiles. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.
4. Achenbach, T.M., & Rescorla, L.A. (2000). Manual for the ASEBA Preschool Forms & Profiles. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.
5. Achenbach, T.M., Becker, A., Döpfner, M., Heiervang, E., Roessner, V., Steinhausen, H.C., & Rothenberger, A. (2008). Multicultural assessment of child and adolescent psychopathology with ASEBA and SDQ instruments: Research findings, applications.
6. Achenbach, T.M., et al. (2007). Multicultural Supplement to the Manual for the ASEBA School-Age Forms & Profiles. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.

18.2 Key Research References

7. Achenbach, T.M. (2009). The Achenbach System of Empirically Based Assessment (ASEBA): Development, Findings, Theory, and Applications. Burlington, VT: University of Vermont, Research Center for Children, Youth, & Families.
8. Achenbach, T.M., McConaughy, S.H., & Howell, C.T. (1987). Child/adolescent behavioral and emotional problems: Implications of cross-informant correlations for situational specificity. *Psychological Bulletin*, 101(2), 213-232.
9. Ivanova, M.Y., et al. (2007). Testing the 8-syndrome structure of the CBCL in 30 societies. *Journal of Clinical Child and Adolescent Psychology*, 36(3), 405-417.

10. Nakamura, B.J., Ebesutani, C., Bernstein, A., & Chorpita, B.F. (2009). A psychometric analysis of the Child Behavior Checklist DSM-oriented scales. *Journal of Psychopathology and Behavioral Assessment*, 31(3), 178-189.
11. Rescorla, L.A., et al. (2007). Epidemiological comparisons of problems and positive qualities reported by adolescents in 24 countries. *Journal of Consulting and Clinical Psychology*, 75(2), 351-358.
12. Srinath, S., Girimaji, S.C., Gururaj, G., Seshadri, S., Subbakrishna, D.K., Bhola, P., & Kumar, N. (2005). Epidemiological study of child & adolescent psychiatric disorders in urban & rural areas of Bangalore, India. *Indian Journal of Medical Res*

18.3 Indian Validation Studies

13. Malhotra, S., Varma, V.K., & Verma, S.K. (1988). Childhood psychopathology measurement schedule (CPMS): Development and standardization. *Indian Journal of Psychiatry*, 30, 325-331.
14. Singhi, P., & Walia, B.N.S. (1993). Behaviour problems in pre-school children. *Indian Pediatrics*, 30, 475-478.
15. Shenoy, J., Kapur, M., & Kaliaperumal, V.G. (1998). Psychological disturbance among 5 to 8-year-old school children: A study from India. *Social Psychiatry and Psychiatric Epidemiology*, 33, 66-73.

18.4 Online Resources

16. ASEBA Official Website: <https://aseba.org>
17. National Child Traumatic Stress Network (NCTSN) -- CBCL Review: <https://www.nctsn.org/measures/child-behavior-checklist-ages-6-18>
18. ASEBA Research Publications Database: <https://aseba.org/research/>

APPENDIX A: QUICK REFERENCE -- T-SCORE CUTOFFS

At-a-Glance Decision Table

Scale Type	Normal	Borderline	Clinical	Clinical	
Syndrome Scales (8 narrow-band)	T < 65	T = 65-69	T >= 70		
Broad-Band Scales (Int, Ext, Total)	T < 60	T = 60-63	T >= 64		
DSM-Oriented Scales (6 scales)	T < 65	T = 65-69	T >= 70		
Competence Scales (Activities, Social, School)	T > 35	T = 31-35	T < 31		
Total Competence	T > 37	T = 33-37	T < 33		

Key Reminders

- Syndrome & DSM scales: HIGHER T-score = MORE problems
- Competence scales: LOWER T-score = MORE problems (reversed!)
- Borderline is clinically meaningful -- don't dismiss it as "normal"
- Standard Error of Measurement: ~ 2-4 T-score points; differences < 5 points may reflect measurement error
- For treatment tracking: Look for >= 5 T-score point change to infer meaningful improvement

Percentile Equivalents

T-Score	Percentile	Meaning	
50	50th	Average	
55	69th	Slightly above average (more problems)	
60	84th	1 SD above mean	
63	90th	Broad-band clinical cutoff	
65	93rd	Syndrome clinical cutoff	
70	97th	2 SD above mean	
75	99th	Extremely elevated	

APPENDIX B: ITEM-TO-SCALE ASSIGNMENT SUMMARY

Syndrome Scale Item Assignments (CBCL/6-18, 2001 Edition)

Note: Item numbers refer to the CBCL/6-18 form. Some items appear on more than one scale.

I. Anxious/Depressed (13 items)

Items: 14, 29, 30, 31, 32, 33, 35, 45, 50, 52, 71, 91, 112

II. Withdrawn/Depressed (8 items)

Items: 5, 42, 65, 69, 75, 102, 103, 111

III. Somatic Complaints (11 items)

Items: 47, 49, 51, 54, 56a, 56b, 56c, 56d, 56e, 56f, 56g

IV. Social Problems (11 items)

Items: 11, 12, 25, 27, 34, 36, 38, 48, 62, 64, 79

V. Thought Problems (15 items)

Items: 9, 18, 40, 46, 58, 59, 60, 66, 70, 76, 83, 84, 85, 92, 100

VI. Attention Problems (10 items)

Items: 1, 4, 8, 10, 13, 17, 41, 61, 78, 80

VII. Rule-Breaking Behavior (17 items)

Items: 2, 26, 28, 39, 43, 63, 67, 72, 73, 81, 82, 90, 96, 99, 101, 105, 106

VIII. Aggressive Behavior (18 items)

Items: 3, 16, 19, 20, 21, 22, 23, 37, 57, 68, 86, 87, 88, 89, 94, 95, 97, 104

Broad-Band Scale Composition

| Broad-Band Scale | Component Syndrome Scales | Note |
| Internalizing | Anxious/Depressed + Withdrawn/Depressed + Somatic Complaints | Items on multiple Internalizing scales counted on
| Externalizing | Rule-Breaking + Aggressive Behavior | Items on both scales counted only once |
| Total Problems | ALL 113 items | Includes ~17 items not on any syndrome scale |

Items NOT Assigned to Any Syndrome Scale

Approximately 17 items contribute to Total Problems but are not assigned to any of the 8 syndrome scales. These include items that did not load consistently on any single factor or that represent relatively independent problems. They still contribute

DSM-Oriented Scale Item Assignments

Affective Problems (13 items)

Items: 5, 14, 18, 24, 35, 47, 49, 51, 52, 54, 76, 77, 100, 102, 103

Anxiety Problems (6 items)

Items: 11, 29, 30, 45, 50, 112

Somatic Problems (7 items)

Items: 47, 49, 51, 54, 56a, 56b, 56c, 56f, 56g

ADHD Problems (7 items)

Items: 4, 8, 10, 13, 17, 41, 78

Oppositional Defiant Problems (5 items)

Items: 3, 22, 23, 86, 95

Conduct Problems (15 items)

Items: 15, 16, 19, 20, 21, 26, 28, 37, 39, 43, 63, 82, 90, 97, 101

APPENDIX C: CLINICAL TIPS FOR INTERPRETATION

C.1 Common Interpretation Mistakes to Avoid

| Mistake | Why It's Wrong | Correct Approach |
| "The CBCL shows the child HAS ODD" | CBCL does not diagnose | "The CBCL profile is consistent with ODD and warrants diagnostic e
| Ignoring borderline scores | Borderline = clinically meaningful | Always report and discuss borderline elevations |
| Interpreting one informant as "correct" | All informants offer valid perspectives | Report each informant's data; discuss agreem
| Using only Total Problems score | Masks specific patterns | Always examine the full profile -- specific elevations guide interve
| Ignoring competence data | Problems without context | Competence provides resilience/vulnerability context |
| Interpreting a single elevation as definitive | May reflect one unusual day | Consider base rates, pattern of elevations, and cl
| Ignoring items scored "2" on critical items | Even if scale is not elevated | Always individually review critical items for safe

C.2 Red Flags Requiring Immediate Attention

Regardless of scale scores, the following require immediate clinical follow-up:

Item Content	Action Required
Talks about killing self (Item 91) scored 1 or 2	Immediate suicide risk assessment
Deliberately harms self (Item 18) scored 1 or 2	Self-harm safety assessment
Hears sounds/voices (Item 40) scored 2	Evaluate for psychotic symptoms
Sees things that aren't there (Item 70) scored 2	Evaluate for psychotic symptoms
Sets fires (Item 72) scored 1 or 2	Fire-setting risk assessment
Cruel to animals (Item 15) scored 2	Evaluate conduct disorder severity and risk
Runs away from home (Item 67) scored 2	Safety planning
Uses drugs/alcohol (Items 2, 105) scored 1 or 2	Substance use evaluation

C.3 Integrating CBCL with Other Assessment Data

Other Data Source	How to Integrate with CBCL
Clinical Interview	CBCL generates hypotheses; interview confirms/disconfirms
Behavioral Observation	Verify whether observed behavior matches parent report
Teacher Report Form (TRF)	Cross-setting comparison; agreement = pervasive; discrepancy = context-specific
Youth Self-Report (YSR)	Especially important for internalizing -- youth often reports internal states better
IQ/Cognitive Testing	Context for academic difficulties on School scale
Projective Tests	Depth understanding of internal dynamics underlying CBCL patterns
Developmental History	Context for onset, course, and risk/protective factors

C.4 Interpreting Change Over Time

When comparing two CBCL administrations (e.g., pre/post treatment):

Change in T-Score	Interpretation
≥ 5 points decrease	Likely meaningful improvement (exceeds SEM)
3-4 points decrease	Possible improvement; within measurement error range
0-2 points change	No meaningful change
Movement from Clinical $\rightarrow$ Normal range	Clinically significant change (even if T-score change is small near cutoff)
Remains Clinical but ≥ 10 point drop	Significant improvement despite remaining in clinical range

Reliable Change Index (RCI): A change is statistically reliable ($p < .05$) when it exceeds $1.96 \times \text{SEM} \times \sqrt{2}$. For most CBCL scales, this is approximately 7-10 raw score points or 5-7 T-score points.

C.5 Cultural Considerations for Indian Clinical Practice

| Consideration | Guidance |
| Somatic Complaints | May be higher in Indian populations as culturally normative expression of distress; interpret elevation wit
Social expectations	"Won't talk" or "withdrawn" may be interpreted differently where child deference to elders is expected
Academic pressure	School scale should be interpreted in context of Indian academic expectations
Gender norms	Some externalizing behaviors may be seen differently for boys vs. girls in Indian cultural context
Joint family context	Multiple informants available (parents, grandparents); consider whose perception is reported
Norms	Use multi-society norms (2007) when available; interpret US norms with cultural sensitivity

C.6 Specific Clinical Scenarios and CBCL Interpretation

Scenario: Suspected ADHD

- Key scales: Attention Problems (Syndrome) + ADHD Problems (DSM)
- Must also check: Rule-Breaking, Aggressive Behavior (comorbidity)
- Required: TRF comparison (must be elevated in school setting too)

- Interpretation: Attention Problems T >= 70 on BOTH CBCL and TRF strongly supports ADHD hypothesis

Scenario: Suspected Depression

- Key scales: Withdrawn/Depressed, Anxious/Depressed + Affective Problems (DSM)
- Must also check: Somatic Complaints, School scale (academic decline)
- Note: Internalizing broad-band will be elevated
- Interpretation: Combined Withdrawn/Depressed + Anxious/Depressed in clinical range with low competence suggests depressive disorder

Scenario: Suspected Anxiety Disorder

- Key scales: Anxious/Depressed + Anxiety Problems (DSM)
- Must also check: Somatic Complaints (anxiety somatization), Social Problems (social anxiety)
- Note: Withdrawn/Depressed may also be elevated if avoidance is prominent
- Interpretation: Anxiety Problems DSM scale >= 70 with Somatic Complaints elevation suggests anxiety with somatic expression

Scenario: Suspected Conduct Disorder

- Key scales: Rule-Breaking + Aggressive Behavior + Conduct Problems (DSM)
- Must also check: Attention Problems (ADHD comorbidity), Social Problems (peer rejection)
- Note: Externalizing broad-band will be elevated
- Critical items: Cruelty, fire-setting, stealing, truancy -- check individual item scores
- Interpretation: Conduct Problems DSM scale >= 70 with multiple critical items scored "2" = high concern

Scenario: Suspected Autism Spectrum Disorder

- Key scales: Social Problems, Thought Problems, Withdrawn/Depressed
- Must also check: Attention Problems (comorbidity common)
- Note: CBCL is NOT an ASD-specific tool -- use as supplement to ASD measures (ADOS, ADI-R, SRS)
- Interpretation: Combined Social Problems + Thought Problems + Withdrawn pattern may suggest ASD features warranting specific assessment

APPENDIX D: COMPARISON OF CBCL VERSIONS

D.1 CBCL/6-18 vs. CBCL/1.5-5

Feature	CBCL/6-18 (School-Age)	CBCL/1.5-5 (Preschool)
Age range	6-18 years	1.5-5 years
Number of problem items	113	99
Response scale	Same (0, 1, 2)	Same (0, 1, 2)
Time frame	Past 6 months	Past 2 months
Competence scales	Activities, Social, School	Not included
Language assessment	Not included	Language Development Survey (LDS) included
Syndrome scales	8 scales	7 scales
Internalizing syndromes	Anxious/Depressed, Withdrawn/Depressed, Somatic Complaints	Emotionally Reactive, Anxious/Depressed,
Externalizing syndromes	Rule-Breaking, Aggressive Behavior	Attention Problems, Aggressive Behavior
Neither syndromes	Social Problems, Thought Problems, Attention Problems	Sleep Problems
DSM-oriented scales	6 scales	5 scales
Broad-band scales	Internalizing, Externalizing, Total	Same structure
Companion teacher form	TRF/6-18	C-TRF/1.5-5
Self-report	YSR/11-18 (ages 11+)	Not available (too young)

D.2 CBCL/6-18 vs. TRF/6-18 vs. YSR/11-18

Feature	CBCL/6-18	TRF/6-18	YSR/11-18
Informant	Parent	Teacher	Youth (self)
Age range	6-18	6-18	11-18
Problem items	113	113	112
Competence items	20 (Activities, Social, School)	Adaptive functioning items	Competence items (same as CBCL)
Unique content	--	Academic performance ratings by teacher	Socially desirable items added
Syndrome scales	Same 8 scales	Same 8 scales	Same 8 scales
Clinical cutoffs	Same	Same	Same
Cross-informant comparison	Compared with TRF and YSR	Compared with CBCL and YSR	Compared with CBCL and TRF
Best for assessing	Home behavior; parent perspective	School behavior; academic focus	Internal states; self-perception
Typical agreement with CBCL	--	r ~ .28-.44	r ~ .25-.48

D.3 Key Development Changes Across CBCL Editions

Feature	1983 Edition	1991 Edition	2001 Edition (Current)
Syndrome derivation	Separate scales for each age-sex group	Unified cross-sex scales	Refined unified scales
Number of syndromes	Varied by age-sex	8 core syndromes	Same 8 + 6 DSM-oriented
DSM alignment	None	None	6 DSM-oriented scales
Norms	1,442 (US)	2,368 (US)	1,753 (US) + multicultural
Computer scoring	Not available	Available	Enhanced ASEBA-ADM
Cross-cultural data	Limited	Growing	Extensive (31 societies)
Research base	Emerging	Substantial	Massive (10,000+ studies)

D.4 Choosing the Right Version

Clinical Situation	Version to Use
Child aged 2-5, parent report	CBCL/1.5-5
Child aged 6-18, parent report	CBCL/6-18
Child aged 6-18, teacher report	TRF/6-18
Adolescent aged 11-18, self-report	YSR/11-18
Preschooler in daycare, caregiver report	C-TRF/1.5-5
Multi-informant assessment (full battery)	CBCL + TRF + YSR (if >=11)
Treatment outcome tracking	Same version at pre and post
Cross-cultural research	Use official translated version + multi-society norms

APPENDIX E: FREQUENTLY ASKED QUESTIONS (FAQ)

For MPhil Trainees

- Q1: Can I photocopy the CBCL form for clinical use?
- Q2: What if a parent rates an item as "1.5" or between two values?
- Q3: What if more than 8 items are left blank?
- Q4: Can grandparents or other relatives fill out the CBCL?
- Q5: Should I use the 1991 or 2001 edition?
- Q6: How do I score items that appear on multiple syndrome scales?
- Q7: What if the child is exactly 6 years old -- which version?
- Q8: Can the CBCL be used for children with intellectual disability?
- Q9: How often can the CBCL be re-administered?
- Q10: Is there an Indian-normed version of the CBCL?
- Q11: What if parent and teacher CBCL/TRF profiles are completely different?
- Q12: Can I use the CBCL in forensic/custody evaluations?

FINAL NOTES FOR TRAINEES

How to Study This Guide

1. First pass: Read through all sections to understand the overall framework
2. Practice scoring: Obtain sample protocols and practice hand-scoring
3. Practice interpretation: Work through the profile patterns in Section 13 with real or practice cases
4. Practice report writing: Use the template in Section 15 for your first reports
5. Master the cutoffs: Memorize the quick reference in Appendix A
6. Supervision: Discuss your first 5-10 CBCL reports with your supervisor before writing independently

Key Points to Remember for Exams and Vivas

1. CBCL is empirically-based (bottom-up from data), not theory-driven (top-down)

2. Developer: Achenbach & Edelbrock (1983); Current revision: Achenbach & Rescorla (2001)
3. Part of ASEBA (Achenbach System of Empirically Based Assessment)
4. 8 syndrome scales + 2 broad-band + 6 DSM-oriented + 3 competence + Total Problems
5. Cutoffs: Syndrome scales ≥ 70 = Clinical; Broad-band ≥ 64 = Clinical
6. Competence cutoffs are REVERSED -- lower = worse
7. Multi-informant comparison is essential (CBCL + TRF + YSR)
8. Does NOT provide diagnoses -- provides dimensional scores and diagnostic hypotheses
9. 113 problem items rated 0-1-2 for past 6 months
10. Validated in 100+ cultures and translated into 100+ languages

This guide was prepared as a comprehensive clinical reference for MPhil Clinical Psychology trainees. It is based on published manuals, peer-reviewed research, and clinical training standards. For clinical use, always refer to the original ASEBA manu

Last updated: 2024

END OF GUIDE